5.12 Autonomic Dysreflexia

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I have a pounding headache." **Objective** • Injury at T6 or higher • BP 200/110 mmHg, HR 50/min • Flushed and sweating above the level of injury	**ACTUAL** **Autonomic dysreflexia** related to noxious stimulus below the injury level as evidenced by severe hypertension, bradycardia and headache.	BP back to baseline once the trigger is removed.	• Sit him up and lower his legs. • Loosen anything tight he is wearing. • Check the bladder (kinked catheter, full bladder) and the bowel (impaction). • BP every 2–5 minutes. • If BP stays high, give the antihypertensive as prescribed.	BP normal: **goal met**.
Subjective • "What caused this?" **Objective** • First time this has happened • Cannot name what sets it off	**ACTUAL** **Deficient knowledge** related to prevention of dysreflexia as evidenced by questions.	Patient and family can name the triggers and what to do first.	• Teach the usual triggers: full bladder, constipation, tight clothes, pressure sores. • Teach the emergency steps: sit up, loosen clothing, check the bladder. • Give him an emergency card.	Explains correctly: **goal met**.
Subjective • *No specific complaint* **Objective** • Spinal cord injury at T4	**RISK** **Risk for autonomic dysreflexia** as evidenced by risk factors: spinal cord injury at T6 or above.	Episodes prevented, or treated straight away.	• Keep bladder and bowel routines going. • Check skin and clothing for anything pressing. • Teach the family the emergency steps.	No episodes: **goal met**.